ANNEX I

SUMMARY OF PRODUCT CHARACTERISTICS

1. NAME OF THE MEDICINAL PRODUCT

ELIQUIS 2.5 mg film-coated tablets

2. QUALITATIVE AND QUANTITATIVE COMPOSITION

Each film-coated tablet contains 2.5 mg apixaban.

Excipients:
Each film-coated tablet contains 51.43 mg lactose (see section 4.4).

For a full list of excipients, see section 6.1.

3. PHARMACEUTICAL FORM

Film-coated tablet
Yellow, round tablets debossed with 893 on one side and 2½ on the other side.

4. CLINICAL PARTICULARS

4.1 Therapeutic indications

Prevention of venous thromboembolic events (VTE) in adult patients who have undergone elective hip or knee replacement surgery.

4.2 Posology and method of administration

Posology
The recommended dose of ELIQUIS is 2.5 mg taken orally twice daily. The initial dose should be taken 12 to 24 hours after surgery.

Physicians may consider the potential benefits of earlier anticoagulation for VTE prophylaxis as well as the risks of post-surgical bleeding in deciding on the time of administration within this time window.

In patients undergoing hip replacement surgery:
The recommended duration of treatment is 32 to 38 days.

In patients undergoing knee replacement surgery:
The recommended duration of treatment is 10 to 14 days.

If a dose is missed, the patient should take ELIQUIS immediately and then continue with twice daily intake as before.

Switching treatment from parenteral anticoagulants to apixaban (and vice versa) can be done at the next scheduled dose (see section 4.5).

Renal impairment
Because there is no clinical experience in patients with creatinine clearance < 15 ml/min, or in patients undergoing dialysis, apixaban is not recommended in these patients (see sections 4.4 and 5.2). Limited clinical data in patients with severe renal impairment (creatinine clearance 15-29 ml/min) indicate that apixaban plasma concentrations are increased in this patient population, therefore, apixaban is to be used with caution in these patients (see sections 4.4 and 5.2).

No dose adjustment is necessary in patients with mild or moderate renal impairment (see section 5.2).

Hepatic impairment
ELIQUIS is contraindicated in patients with hepatic disease associated with coagulopathy and clinically relevant bleeding risk (see section 4.3).

It is not recommended in patients with severe hepatic impairment (see sections 4.4. and 5.2).

It should be used with caution in patients with mild or moderate hepatic impairment (Child Pugh A or B). No dose adjustment is required in patients with mild or moderate hepatic impairment (see sections 4.4 and 5.2).

Patients with elevated liver enzymes (ALT/AST >2 x ULN) or total bilirubin ≥1.5 x ULN were excluded in clinical trials. Therefore ELIQUIS should be used with caution in this population (see sections 4.4 and 5.2). ALT should be measured as part of the standard pre-operative evaluation (see section 4.4).

Body weight
No dose adjustment required (see section 5.2).

Gender
No dose adjustment required (see section 5.2).

Elderly
No dose adjustment required (see sections 4.4 and 5.2).

Paediatric population
The safety and efficacy of ELIQUIS in children below age 18 have not been established. No data are available.

Method of administration
Oral use.
ELIQUIS should be swallowed with water, with or without food.

4.3 Contraindications

- Hypersensitivity to the active substance or to any of the excipients.
- Clinically significant active bleeding.
- Hepatic disease associated with coagulopathy and clinically relevant bleeding risk (see section 5.2)

4.4 Special warnings and precautions for use

Haemorrhage risk
As with other anticoagulants, patients taking ELIQUIS are to be carefully observed for signs of bleeding. It is recommended to be used with caution in conditions with increased risk of haemorrhage, such as: congenital or acquired bleeding disorders; active ulcerative gastrointestinal disease; bacterial endocarditis; thrombocytopenia; platelet disorders; history of haemorrhagic stroke; severe uncontrolled hypertension; and recent brain, spinal, or ophthalmological surgery. ELIQUIS administration should be discontinued if severe haemorrhage occurs (see sections 4.8 and 4.9).

Renal impairment

6. PHARMACEUTICAL PARTICULARS

6.1 List of excipients

Tablet core:
Anhydrous lactose
Microcrystalline cellulose (E460)
Croscarmellose sodium
Sodium laurilsulfate
Magnesium stearate (E470b)

Film coat:
Lactose monohydrate
Hypromellose (E464)
Titanium dioxide (E171)
Triacetin (E1518)
Yellow iron oxide (E172)

6.2 Incompatibilities

Not applicable.

6.3 Shelf life

3 years

6.4 Special precautions for storage

This medicinal product does not require any special storage condition.

6.5 Nature and contents of container

Alu-PVC/PVdC blisters of 10 film-coated tablets. Cartons of 10, 20 and 60.

Alu-PVC/PVdC perforated unit dose blisters of 60 x 1 film-coated tablet or 100 x 1 film-coated tablet.

Not all pack sizes may be marketed.

6.6 Special precautions for disposal

No special requirements.

7. MARKETING AUTHORISATION HOLDER

Bristol-Myers Squibb/Pfizer EEIG, Bristol-Myers Squibb House,
Uxbridge Business Park, Sanderson Road, Uxbridge, Middlesex
UB8 1DH
United Kingdom

8. MARKETING AUTHORISATION NUMBER(S)

9. DATE OF FIRST AUTHORISATION/RENEWAL OF THE AUTHORISATION

Date of first authorisation:

10. DATE OF REVISION OF THE TEXT

Detailed information on this product is available on the website of the European Medicines Agency http://www.ema.europa.eu

ANNEX II

A. MANUFACTURING AUTHORISATION HOLDER(S) RESPONSIBLE FOR BATCH RELEASE

B. CONDITIONS OF THE MARKETING AUTHORISATION